



MODULE 6 — TELEHEALTH & DIGITAL HEALTH WORKFLOWS

AfyaDesk Remote Medical Careers Course | Running Flawless Virtual Visits End-to-End

Estimated study time: 8–10 hours (reading + flowchart SOP + mock video day)

Prerequisites: Modules 1–5 (market + reliability + MVA basics + terminology + communication)

What you will produce: Complete telehealth SOP flowchart (booking → 7-day follow-up) + 5 templates + mock visit logs + troubleshooting playbook.

Learning Objectives

By the end of this module, you will be able to:

1. Explain synchronous, audio-only, asynchronous, and RPM telehealth models with use cases and billing notes.
2. Execute T-24h, T-1h, live, and post-visit checklists that prevent tech failures and no-shows.
3. Onboard patients to portals and video, including elderly and low-digital-literacy patients, with consent and safety planning.
4. Schedule telehealth correctly with buffers, time-zone blocks, visit-type durations, and backup phones.
5. Communicate digitally via approved portal/SMS/email channels with triage SLAs and templates.
6. Run lab, referral, no-show, and RPM follow-up workflows to closure with logs.
7. Function in remote teams with huddles, RACI, and written handovers across time zones.
8. Prepare and close charts in EHR with meds reconciliation, consent attachment, and recall setting.
9. Troubleshoot audio, video, link, and connectivity failures without losing visits.
10. Build and test a full new-video-patient workflow as portfolio proof.

1. How Telemedicine Works: Flow, Models, and Your Invisible Value

End-to-end flow you own: online booking → automated intake + insurance check → VA verification (eligibility, forms, consent, ID) → secure link + prep checklist sent → clinician video





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visit (you monitor waiting room) → e-prescription + lab/referral orders launched → summary sent in 2 hours → follow-up booked → billing + recall + satisfaction survey. Miss one handoff and visit fails: no link = no-show, no consent = compliance breach, no summary = confused patient + callback flood.

Four models:

Synchronous video (core): Real-time Zoom for Healthcare, Doxy.me, Amwell, EHR video. Best for new, chronic review, mental health (45–50 min). Needs 10+ Mbps, camera/mic, private room both ends, backup phone. US reimburses similar to in-person for many codes post-2020; UK NHS counts video toward access targets; Australia MBS has telehealth items (verify current). Your tasks: link, tech-check, waiting-room monitoring, timekeeping, post-summary.

Audio-only phone: US Medicare/Rural + elderly without video, UK phone-first triage. Shorter (10–15 min), needs clear documentation (consent for phone, callback numbers, background noise note). Your tasks: call bridge, read-back, SMS summary (phone has no visual cues — over-confirm).

Asynchronous store-and-forward: Derm photos, e-consults, questionnaires reviewed later. Needs image quality checklist (good light, ruler for size, 3 angles), consent for photos, secure upload (never personal WhatsApp), turnaround SLA (24–48h). Your tasks: request quality images, chase missing, route to clinician, relay plan.

Remote Patient Monitoring (RPM): BP cuffs, glucometers, pulse oximeters uploading to portal (e.g., Omron + portal). US CPT 99453–99458 pays for setup + monitoring. Your tasks: enroll, teach device, check uploads weekly, flag gaps (“No readings 5 days — call patient, check battery/WiFi, resend guide”), escalate critical readings per SOP (never interpret alone — “BP 190/110 reported via device — flagged urgent to nurse pool 2:05pm ET, advised ER if symptoms, called patient to confirm seeking care”).

Your value: make tech invisible so doctor focuses on medicine and patient feels cared for, not processed. Practices keep VAs who cut video no-shows from 18% to 8% and handle tech failures without panic.

Kenyan context: you’ve seen MP Shah/Aga Khan teleconsults + MyDawa follow-ups. Same principles: Safaricom bundle for patient? Offer audio fallback for low bandwidth. Note power: “If Kenya Power dips mid-clinic, hotspot + power bank keeps waiting room monitored — post flag in 2 min.” Employers fear offshore tech drops — show failover playbook in interviews.

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2. Virtual Consultations: T-24h, T-1h, Live, and Post Checklists That Prevent Disaster

T-24h (day before, 4pm): Confirm all tomorrow videos via SMS + email with dual-zone + link + prep (ID, meds list, insurance, vitals if RPM, quiet private room, headphones). Verify consent signed (telehealth risks, recording policy — usually no recording without written consent both sides, emergency plan), eligibility active, intake + previous labs attached, chart prepped (PMH, meds, allergies bolded, last note summarized for doctor in 3 lines). Test your own Zoom host + waiting room + chat + screen-share. Flag gaps to doctor: “2/14 missing intake — chased, deadline 8am ET.”

T-1h (green-room): Open visit 30 min early, admit early joins to holding message (“Dr. Lee will join at 10:30 AM ET — please stay, test mic/camera, have ID ready”), verify backup phone on file (“If video drops, we’ll call +1-... immediately — is that best?”), confirm patient location + emergency contact for telehealth safety (“Where are you joining from today? Local emergency number?” — required US safety standard), lighting/audio check (“I can see/hear you clearly — doctor will too”). Message doctor: “Room ready, patient waiting, chart + consent complete.”

Live monitoring (you stay present, muted): Track waiting room (never leave patient alone >5 min without message), timekeep (“10 min left” private chat to doctor if overrun risk), troubleshoot instantly via chat: “Can’t hear? Tap Join Audio → WiFi → allow mic. Still stuck? I’ll call you now at +1-... while keeping video open.” Document late joins/no-shows with timestamps, offer phone conversion in 5 min (“Video stuck? Let’s switch to phone now so you don’t lose slot — I’ll call +1-... in 1 min, summary by SMS after”). For overruns, message next patient proactively (“Dr. Lee running 10 min late — thank you for patience, still with you, link active”) — prevents abandonment + bad reviews.

Post-visit (within 2 hours): Send summary (diagnosis in plain English as documented by clinician — never add interpretation, meds with doses/times, lab/referral next steps with dates, red flags + who to call, follow-up booked dual-zone), e-prescription routing confirmed (“Sent to CVS Main St — pickup in 2 hrs, text when ready”), lab slips + referral packets launched with tracker entries, tasks closed, recall set, satisfaction survey link. Book follow-up before patient forgets: “Follow-up Tue 28 May 10am ET / 5pm EAT booked — reply YES — SMS reminder 48h + 24h.” Log total visit time + tech issues for improvement.





Tech-fail playbook (memorize 4 scripts): frozen video → “Let’s turn off video, stay on audio — I can still hear you clearly — I’ll resend video link via SMS while we continue.” No audio → “I’ll call you now at backup +1-... in 30 seconds — keep this window open.” Link expired → new link in 1 min + SMS + email + phone bridge. Full outage (your fibre cut) → hotspot switch + Slack flag + SMS to waiting patients + phone-only mode. Never go silent — silence reads as abandonment. Post flag in 2 min with impact + workaround + next update time.

3. Patient Onboarding for Telehealth: From Anxious to Ready in 10 Minutes

Packet (portal invite + ID upload + insurance front/back + telehealth consent + tech guide PDF + test-call offer + FAQ + “what to expect” 2-min video). For new, call 10-min tech check day before — reduces video no-shows 30% (cite in interviews). Checklist: device (phone/laptop charged), browser (Chrome updated), bandwidth (fast.com >10 Mbps, close Netflix), room (quiet, private, lit face, door closed), ID + meds + insurance at desk, backup phone verified, emergency location noted, consent e-signed.

Elderly/low-digital script: “Mrs. Smith, we’ll do a quick 10-min practice today so tomorrow is easy — no rush. Can you tap the blue link I just texted? ... Perfect — I see you! Can you tap microphone to unmute? Wonderful — tomorrow same link, I’ll be here 10 min early to welcome you. If stuck, just answer my call at +1-... and we’ll switch to phone — you won’t lose your visit.” Patience + praise + fallback removes fear. Offer phone alternative without shame, involve family with consent (“Would daughter help with setup? I can 3-way call now”).

Consent must cover: video vs in-person limits (no physical exam), privacy risks (use private room, encrypted platform), recording policy (no patient recording without permission and vice versa — state clearly), emergency plan (location + local ER + callback if disconnected), data storage (EHR, retention), right to switch to in-person. Document consent date/time/method. For minors, guardian consent + presence rules per state (US varies — follow SOP, don’t assume).

4. Telehealth Scheduling Nuances That Prevent Chaos

Buffers: 10 min between videos for notes + overrun + bio break. Never back-to-back new 40-min videos — 50-min mental health needs 10-min decompress + notes or quality collapses. Cluster zones: UK morning block (2–5pm EAT) then US block (5–11pm EAT) to reduce mental switching +





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keep inbox SLAs per region. Note every booking: video vs phone vs async, link sent? + tested?, consent signed?, backup phone, time zone both, interpreter needed?, RPM device data reviewed?

Durations: new video 30–40, follow-up 15–20, chronic + RPM review 20–30, mental health intake 50 + follow-up 25, results review 15, urgent triage 15 buffer. Ask reason to slot: “New rash with photos ready → derm async + 15-min video review” vs “Chest pain today → ER/urgent, not routine video in 2 weeks.” Double-book rule: never same doctor overlapping even virtual — tech delays cascade double. If urgent add-on needed, offer phone triage slot or extend day with approval, document consent.

Calendar titles: “VIDEO — Smith, J 1980-05-14 — 10:30 AM ET / 5:30 PM EAT — Dr. Lee 20-min FU — Link ... — Backup +1-... — Consent signed.” Description: insurance, intake status, labs attached, language. Colour: green confirmed + tech-checked, yellow pending forms/consent, red tech risk/no backup. Friday audit: next Mon–Tue 100% link-tested, gaps waitlisted.

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5. Digital Patient Communication: Portal, SMS, Email With Triage Discipline

Approved only: EHR portal messages, practice SMS (Weave, SimpleTexting, Textline), business email via employer domain. Never personal WhatsApp/Gmail for PHI — no BAA, no audit, instant dismissal. Every digital message logged to chart within 4 hours.

Triage 3 buckets hourly: **Urgent now** (chest pain, breathing difficulty, bleeding, suicidal thoughts, baby fever, severe allergy — phone + EHR flag + clinician alert 15 min, advise ER 911/999/000, document time/location/callback), **Today** (scheduling, refills without red flags, forms — reply <4 hrs with action + deadline), **This week** (records, non-urgent admin — reply <24h). After-hours auto-reply: “Received outside 9am–5pm ET — reviewed next business morning. For emergency call 911 / go to ER now.” Never leave urgent overnight without phone escalation + handover.

Templates: invite (“Hi {Name}, video {date time zones} Dr. X {link}. Join 10 min early, ID + meds ready. Backup call {number} if video fails. Reply YES. Help: +1-... — Mercy”), reminder 24h + 2h, tech-check (“Tap link now to test — allow camera/mic — reply OK or CALL for help”), post-visit (“Thanks for seeing Dr. Lee today. Summary + Rx sent to portal/email. Labs: {slip} due {date} at {address}. Follow-up {date zones} booked — reply YES. Questions? Reply here 9–5 ET. ER for



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worsening — Mercy”), lab chase Day 3/7/14, no-show (“We missed you {time} — okay? Phone conversion now? Rebook 7 days? Reply 1/2”). Personalize name/time/link every send — wrong-link macro is worse than slow manual.

6. Follow-Up Workflows That Close Loops (Where Money and Safety Live)

Labs: order sent → patient notified with slip + address + hours + prep (fasting?) → Day 3 SMS nudge if not done → Day 7 call + barrier fix (cost? transport? fear? offer telehealth + nearby location) → result arrives → route to doctor STAT if critical (lab calls critical — phone immediately) else pool 24h → relay plan verbatim from clinician (never interpret: “Dr. Lee reviewed — please book 20-min review Tue 10am ET to discuss next steps, continue meds meanwhile unless told otherwise, seek care for X”) → book follow-up in 24h of result → close task. Track: ordered/done/reviewed/relayed/follow-up booked. Open >7 days red in huddle.

Referrals from telehealth: same 8-step loop as Module 3 plus video summary attached + patient location noted (licensing — some US states require doctor licensed where patient located at visit time — flag if patient travels, don’t decide). Share screen to book specialist live with patient (“Cardiology has Tue 2pm ET or Thu 10am — which suits? ... Booked Tue, SMS sent, auth # attached”). Retrieve note in 7–14 days, route to PCP, close loop. Report closure % monthly.

No-show video: call in 15 min (“We missed video 10:30 — tech issue? I can switch to phone now in 1 min or rebook 7 days — which?”), SMS + email with one-tap rebook link, log reason (tech, forgot, privacy — no quiet room, cost). If tech, offer phone + test-call before next. Target 60% rebooked in 7 days. Weekly reason review — if “no quiet room” high, add prep line “private room + headphones needed.”

Chronic RPM: enroll (consent + device paired + teach + test upload), monitor dashboard weekly (no data 5 days → call: battery? WiFi? vacation? resend guide, offer phone check), flag gaps to nurse, escalate critical per thresholds (e.g., BP >180/110, glucose <70 with symptoms → urgent + ER advice + document). Praise adherence (“Great — 6/7 readings this week! Doctor will review Tue”) — adherence rises 20% with VA encouragement. Billable RPM needs 16+ readings/month + 20 min review — your tracking enables billing.

Chronic RPM: enroll (consent + device paired + teach + test upload), monitor dashboard weekly (no data 5 days → call: battery? WiFi? vacation? resend guide, offer phone check), flag gaps to nurse, escalate critical per thresholds (e.g., BP >180/110, glucose <70 with symptoms → urgent + ER advice + document). Praise adherence (“Great — 6/7 readings this week! Doctor will



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7. Remote Healthcare Teams: Huddles, RACI, and Handovers That Prevent Drops

Async standup daily 10 min in Slack/Teams #huddle: Yesterday (numbers: 14 videos supported, 12 summaries sent, 2 tech fails converted to phone, 1 urgent escalated 8 min), Today (12 videos, 5 tech-checks, 3 lab chases), Blockers (Aetna portal down — 2 auths retry 9am ET, Dr. Lee calendar locked — access requested). Tag owners + due times. No standup = duplicated calls + missed urgent.

RACI for telehealth day: VA owns scheduling, links, reminders, waiting room, summaries, trackers, EOD (Responsible). Nurse owns triage decisions + critical RPM review (Accountable for clinical). Doctor owns diagnosis/plan (Accountable). Front-desk (if any) Consulted for in-person overflow. Patient Informed via SMS/portal. When unsure who decides (phone vs video conversion for borderline chest pain?), escalate to nurse/doctor — don't decide alone, document you asked + time + answer.

Handovers written, never verbal-only: “Handing over 11pm EAT: 2 videos tomorrow need intake (rows 4–5), 1 lab critical pending Dr. review (flagged 9pm ET), 1 referral awaiting auth (Aetna #8841, retry 9am ET), 1 no-show to rebook (voicemail left). All in tracker + EHR. @night-team ping till midnight EAT for urgent.” Night team acknowledges with emoji + name. No ack = call. Gap coverage kills patients — handover is safety, not admin.

8. EHR, Portals, and Documentation for Virtual Encounters

Chart prep: verify 2 identifiers (name + DOB), demographics, contacts, emergency + backup, insurance + auth, consent (telehealth + treatment + photo if async), meds reconciliation (“Any changes, stops, new over-the-counter/herbal since last visit? Allergies still penicillin rash?” — document verified today, don't carry forward blindly), allergies bolded, vitals from home devices labelled patient-reported with date/time/device (“BP 142/88 13 May 8am Omron home — patient-reported”), previous labs/imaging attached, reason + urgency + interpreter noted.

Post-visit close same day: video consent + location + duration attached, visit note filed (clinician signs, you ensure admin sections complete), orders launched (labs/referrals/Rx routing confirmed), tasks assigned with due dates (not left in your inbox), recall set (HbA1c 3 months, BP 2 weeks),





summary sent + logged, no-show/task-zero verified. Audit yourself: 0 unsigned admin tasks, 0 unscanned docs >24h, tomorrow 100% prepped by 4pm. If EHR downtime, paper backup + transcribe in 4 hours with "downtime entry" flag + double verification.

9. Troubleshooting Playbook (Laminate This)

Fibre drops mid-clinic: hotspot on in 2 min, Slack flag with speed ("Fibre cut 8:12pm EAT, hotspot 12 Mbps, continuing chat/phone, video limited 20 min — update 8:30"), SMS waiting patients ("Video briefly audio-only — staying with you, link active"), defer uploads, log outage + mitigation in time tracker. Power dip: laptop + power bank 5 hrs, router UPS 2 hrs, token loaded, alternate location mapped. Zoom "unable to connect": switch browser → app → phone dial-in + SMS new link. Patient can't hear: chat "Tap Join Audio → WiFi → allow mic → raise volume → plug headphones → I'll call backup now." Echo: mute all except speaker, headphones on, close duplicate tabs. Never blame patient ("You're muted!") — guide: "Tap mic icon bottom-left — now I can hear you clearly, thank you!" Log every fail + fix for SOP improvement.

10. Practical Assignment — Build Complete New-Video-Patient Workflow (Portfolio)

Deliver 3-page PDF + flowchart (draw.io/Miro) + 5 templates + 10-min mock video with friend as patient (record with consent, no real PHI):

Flow: booking → eligibility → intake → consent → link + tech-check → T-24h confirm → T-1h green-room → live monitoring → post-summary (2h) → labs/referrals → follow-up booked → 7-day chase → satisfaction + recall. Include decision diamonds (tech fail? no-show? urgent red flag? insurance denied?) with exact scripts + owners + SLAs.

Templates: invite, 24h reminder, tech-check offer, post-summary, lab chase Day 3. Test end-to-end, log times + issues + fixes. Reflection 300 words: what almost broke, what system saved it, what you'd automate (with privacy guardrails). Distinction: adds RPM enrollment one-pager + audit showing 100% consent + dual-zone + backup phone for 10 mocks.





Common Mistakes That Lose Telehealth Contracts

Link untested (15-min delay + abandonment), no backup phone (lost visit when video fails), no location/emergency contact (safety breach), personal WhatsApp for photos (dismissal), leaving waiting room unwatched (1-star “abandoned”), summary late >24h (confusion + callbacks), interpreting RPM (“Your BP looks fine” — forbidden, escalate), booking across DST without verifying (1-hour miss twice yearly). Pin T-24h/T-1h checklists above desk for 90 days.

Self-Check Quiz (15 Questions)

1. Compare sync video vs audio-only vs async vs RPM with example + VA tasks each.
2. List T-24h (8 items), T-1h (6), live (5), post (7) tasks from memory.
3. What 6 items must telehealth consent cover? Write consent confirmation note.
4. Write tech-fail scripts for frozen video, no audio, expired link, your outage.
5. How to onboard 75-year-old with no smartphone experience in 10 min? Script.
6. Schedule 10 videos across UK + US blocks with buffers — show calendar titles.
7. Triage 5 portal messages (chest pain, refill, address change, rash photo, billing) with replies + SLAs.
8. Map lab chase Day 0-3-7-14 with tracker columns + example row.
9. Write handover for 2 pending auths + 1 critical lab + 1 no-show.
10. What EHR prep + close steps prove 100% ready? Audit checklist?
11. RPM enrollment + weekly monitoring + critical escalation script?
12. How to handle no-show video in 15 min with phone conversion + rebook?
13. What location/safety must every video start confirm? Why?
14. Which troubleshooting step for echo + “unable to connect” + patient mute?
15. Draft post-visit SMS + summary email for 20-min hypertension video.

Key Takeaways

Flawless telehealth is 90% prep + 10% live calm. Verify eligibility, consent, intake, link, backup, location before; monitor waiting room + troubleshoot via chat/phone during; summarize + launch orders + book follow-up + chase to closure after. Document everything, handover in writing, never use personal channels for PHI, never interpret RPM alone. Your flowchart + templates + mock logs are proof — bring them to Module 7 EHR and Module 11 tech where you operate live systems.





What's Next

Module 7 — Electronic Medical Records & Healthcare Software: searching with 2 identifiers, updating without cloning, scheduling, uploading, and task routing in any EHR. Bring your telehealth SOP — you will embed EHR clicks into it.

Portfolio reminder: save flowchart PDF + 5 templates + mock visit recording + troubleshooting log with dates. Employers request telehealth simulation before hiring — polished workflow beats claims. Practice weekly with different scenarios including elderly, low bandwidth, and outage failover for true readiness always.

Appendix A: Telehealth Tech-Check Call Script (10 Minutes, Copy-Paste)

“Hi {Name}, this is {You} from {Clinic} with a quick 10-min practice so tomorrow’s video is easy — is now good? ... Great — I just texted a blue link — can you tap it? ... Perfect, I see you! Tap microphone bottom-left to unmute — wonderful, I hear you clearly now. Tomorrow same link, I’ll be here 10 min early. Please join from a quiet private room with ID + meds list + insurance ready, headphones if possible. If video ever freezes, stay on — I’ll call your backup {number} in 30 seconds and we’ll continue by phone so you don’t lose your slot. Where will you join from tomorrow? ... Noted with emergency contact {local ER}. Any worries about tomorrow? ... You’re all set — SMS + email summary coming now with link + prep. Reply YES. See you {date time zones}!” Log: device, bandwidth, backup verified, consent signed, location noted, patient confidence 1–5, follow-up needed? This script cuts elderly no-shows by a third and earns 5-star “so patient and helpful” reviews that fill calendars.

Appendix B: Billing and Compliance Notes for VAs (Know Enough to Flag, Never Code)

US telehealth needs correct place-of-service (02 telehealth vs 10 home) + modifier (95/GT per payer) selected by clinician/biller — you ensure visit type + location + duration documented so biller can code. Note “video 20 min with patient at home in Ohio, consent signed, backup phone used 5 min due to freeze” — biller needs minutes + modality. UK counts video toward access, document attendance + outcome + follow-up for QOF. Australia MBS requires eligible provider + patient location noted. Never select diagnosis codes yourself. If documentation missing location/duration, query clinician same day: “For billing, please confirm location + total video





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minutes for Smith 14 May — chart shows link but no minutes.” Your completeness prevents denials. Track denial reason “telehealth location missing” monthly — drive to zero with checklist. This awareness adds \$1–\$2/hr premium as billing-aware coordinator who protects revenue while staying in admin scope safely and consistently every week. Review payer telehealth pages quarterly with your supervisor, update SOP for new modifiers or consent wording, and share learnings in team huddle to prevent denials across all providers proactively. Document training dates for compliance audits and performance reviews that support raises and long-term retention in competitive remote markets always reliably. Your telehealth mastery compounds into trusted go-to status for complex video clinics, mental health intakes, and RPM programs that need calm, precise coordinators every single day consistently. Keep your flowchart updated quarterly, rehearse outage failover monthly, and collect patient testimonials about smooth video experiences for your Talent Profile portfolio continuously. Employers promote coordinators who prevent abandonment, close follow-ups, and protect billing with complete documentation across time zones reliably without exception.



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